



Name: Melody Dawn Dashora | **DOB:** 7/1/1978 | **MRN:** 2000422796 |
PCP: Sarah Uddin | **Legal Name:** Melody Dawn Dashora

Emergency Department - Aug 30, 2026

at Texas Health Frisco Emergency Department



Notes from Care Team



ED Provider Notes by Shannon Kelsey Della Monica at 08/30/26 1946

THFEMER ROOM#: TR02

Chief Complaint

Patient presents with

- Motor Vehicle Crash

Pt reports she was involved in an MVC where she was t-boned by a vehicle going 60mph. She was seen at Baylor Grapevine and had a work up. She report severe headache, numbness to her head and tingling on the left side which started approximately 2 hrs prior to arrival. Pt appears to have some left sided facial droop. Pt reports her face does not feel right.

First Provider Contact Date and Time: 08/30/26 1946

Previous History

Past Medical History:

Diagnosis

Date

- ADHD
- Anaphylactic reaction
- Anemia
from MTHFR Mutation, during pregnancy
- Anxiety disorder
- Asthma
- Bronchitis
February 2020
- Chicken pox
as a child
- Crohn's colitis
- CRPS (complex regional pain syndrome type I)
- Depression
- Difficult intravenous access
- DVT (deep venous thrombosis)
- Endometriosis
- Esophagitis, eosinophilic
- Fibromyalgia
- Fracture, facial bones (HCC)
- Gallbladder disease
- Gallstones
- Gastritis
- Herpes

Herpes Simplex I

- Hypothyroid
- IBS (irritable bowel syndrome)
- Insomnia
- Menorrhagia
- Migraine

Resolved

- MTHFR mutation
- OSA (obstructive sleep apnea)

Resolved per pt

- Osteoarthritis
- Pernicious anemia
- Pneumonia

Febuary 2020

- Protein S deficiency

Resolved per pt

- Pulmonary embolism
- TIA (transient ischemic attack)
- Ulcerative colitis

2007

08/2023

Past Surgical History:

Procedure

Laterality Date

- APPENDECTOMY
- BILATERAL TUBAL LIGATION
- CAUTERY OF CERVIX; LASER ABLATION
- COLONOSCOPY

2014,
2009.
2017,

		2019
•ESOPHAGOGASTRODUODENOSC		12/17/20
OPY		19
•HYSTEROSCOPY		
•SURGICAL	N/A	1/17/202
PROCEDURE/COMMENT		0
<i>Procedure: LAPAROSCOPY WITH CO2 LASER;</i>		
<i>Surgeon: Gebert, Alvin Ray, MD; Location: THP OR;</i>		
<i>Service: OBGYN</i>		
•SURGICAL	N/A	1/17/202
PROCEDURE/COMMENT		0
<i>Procedure: hysteroscopy, D&C, frozen section, lysis of</i>		
<i>adhesion, laser excision of endometriosis, , endometrial</i>		
<i>ablation; Surgeon: Gebert, Alvin Ray, MD; Location:</i>		
<i>THP OR; Service: OBGYN</i>		
•SURGICAL	N/A	3/20/202
PROCEDURE/COMMENT		0
<i>Procedure: COLONOSCOPY; Surgeon: Espinoza,</i>		
<i>Magdalena, MD; Location: THF GI; Service:</i>		
<i>Gastroenterology</i>		
•SURGICAL	Left	7/11/202
PROCEDURE/COMMENT		4
<i>Procedure: LEFT STELLATE GANGLION BLOCK;</i>		
<i>Surgeon: Dott, Daltry, MD; Location: THF OR; Service:</i>		
<i>Pain Management</i>		
•SURGICAL	Left	1/22/202
PROCEDURE/COMMENT		5
<i>Procedure: LEFT STELLGATE GANGLION BLOCK;</i>		
<i>Surgeon: Dott, Daltry, MD; Location: THF OR; Service:</i>		

Pain Management

- SURGICAL N/A 2/26/2025
PROCEDURE/COMMENT
Procedure: C7-T1 ESI; Surgeon: Dott, Daltry, MD;
Location: THF OR; Service: Pain Management
- WISDOM TOOTH EXTRACTION Bilateral

Family History

Problem	Relation	Age of Onset
• Suicide	Cousin	

Social History

Socioeconomic History

- Marital status: Divorced
Spouse name: Not on file
- Number of children: Not on file
- Years of education: Not on file
- Highest education level: Not on file

Occupational History

- Not on file

Tobacco Use

- Smoking status: Former
Current 0.50
packs/day:
Types: Cigarettes

- Smokeless tobacco: Never

- Tobacco comments:

Pt stated she quit on her own and recently begin back in November. Pt stated she smokes 1-2 a month

Vaping Use

- Vaping status: Never Used

Substance and Sexual Activity

- Alcohol use: Not Currently

Comment: Pt stated he last had alcohol "its been years"

- Drug use: Never

Comment: pt denies drug use

- Sexual activity: Not on file

Other Topics Concern

- Not on file

Social History Narrative

- Not on file

Social Drivers of Health

Tobacco Use: Medium Risk (8/30/2026)

Patient History

- Smoking Tobacco Use: Former
- Smokeless Tobacco Use: Never
- Passive Exposure: Not on file

Alcohol Use: Not on file

Financial Resource Strain: Low Risk (8/17/2023)

Overall Financial Resource Strain (CARDIA)

• Difficulty of Paying Living Expenses: Not very hard
Food Insecurity: Food Insecurity Present (2/6/2025)
Received from UT Southwestern Medical Center
Hunger Vital Sign

- Within the past 12 months, you worried that your food would run out before you got the money to buy more.: Often true
- Within the past 12 months, the food you bought just didn't last and you didn't have money to get more.: Often true

Transportation Needs: Unmet Transportation Needs (2/6/2025)

Received from UT Southwestern Medical Center
PRAPARE - Transportation

- Lack of Transportation (Medical): Yes
- Lack of Transportation (Non-Medical): Yes

Physical Activity: Inactive (8/17/2023)

Exercise Vital Sign

- Days of Exercise per Week: 0 days
- Minutes of Exercise per Session: 0 min

Stress: Not on file

Social Connections: Socially Isolated (8/17/2023)

Social Connection and Isolation Panel

- Frequency of Communication with Friends and Family: Once a week
- Frequency of Social Gatherings with Friends and Family: Never
- Attends Religious Services: Never
- Active Member of Clubs or Organizations: No

- Attends Club or Organization Meetings: Never
- Marital Status: Divorced

Intimate Partner Violence: Low Risk (8/28/2026)

Received from Baylor Scott & White Health

Interpersonal Safety

- Feels UN-safe at Home or Work/School: no
- Feels Unsafe: Not on file
- Physical Signs of Abuse Present: no

Depression: Low Risk (8/28/2026)

Received from Baylor Scott & White Health

Depression

- PHQ2 Screening score: 0
- Last PHQ-9 Score: Not on file

Housing Stability: High Risk (2/6/2025)

Received from UT Southwestern Medical Center

Housing Stability Vital Sign

- In the last 12 months, was there a time when you were not able to pay the mortgage or rent on time?: Yes
- In the past 12 months, how many times have you moved where you were living?: 0
- At any time in the past 12 months, were you homeless or living in a shelter (including now)?: No

Utilities: At Risk (2/6/2025)

Received from UT Southwestern Medical Center

AHC Utilities

- Threatened with loss of utilities: Yes

Health Literacy: Not on file

Current Encounter

ED Triage Vitals

BP 08/30/26 **155/99**
1945

Pulse 08/30/26 **98**
1945

Resp 08/30/26 **18**
1945

Temp 08/30/26 **(!) 96.1**
1945 **°F (35.6**
°C)

Temp src 08/30/26 **Oral**
2059

SpO2 08/30/26 **95 %**
1945

Pain 08/30/26 **8**
Level 1945

Oxygen --

Therapy

Flow

(L/min)

Oxygen 08/30/26 **room air**
Therapy 2010

O2

Device

Trauma 08/30/26 **Lvl 2**

Activation Level
1952
BMI 08/30/26 34.7
1945

HISTORY OF PRESENT ILLNESS:

Patient is a 48-year-old female with a history of CRPS in her left hand and prior blood clots (pulmonary and lower extremity, last episode 16 years ago) who presents with persistent and evolving symptoms following a motor vehicle collision (MVC) two days ago. The patient was struck on the passenger side by a vehicle traveling approximately 60 mph while she was making a left turn at 17 mph. She experienced loss of consciousness at the scene and was evaluated at Baylor Hospital, where CT imaging (head, cervical spine, chest, abdomen, pelvis, thoracic and lumbar spine) and x-rays of both knees were performed, reportedly without acute findings. Since the accident, she has developed increasing pain, primarily on the left side (neck, leg, arm), extensive bruising, and new symptoms including scalp numbness, left-sided facial tingling, generalized fatigue, difficulty concentrating, and subjective cognitive slowing. She also reports nausea, blurry vision, and worsening ability to answer questions. The patient describes a sensation of detachment and abnormal visual phenomena when closing her eyes. She has a high pain tolerance due to CRPS but reports that her current pain is unusually severe. She has taken

Toradol, methocarbamol, and tramadol for pain with minimal relief. She denies current use of blood thinners. An independent historian (family member) corroborates the patient's increased difficulty with ambulation and answering questions, noting that these symptoms worsened throughout the day.

External records reviewed: Chart review from Baylor Hospital, 2 days prior, CTs (head, cervical spine, chest, abdomen, pelvis, thoracic and lumbar spine), x-rays of both knees, reportedly without acute findings.

Independent historian: Family member present, corroborates cognitive changes and symptom progression.

Medications: Adderall (for ADHD), Alprazolam (for anxiety), Toradol, methocarbamol, tramadol (recently for pain).

Allergies: Tylenol.

Past Medical History: CRPS in left hand, history of pulmonary and lower extremity blood clots (last episode 16 years ago), ADHD, anxiety.

Surgical History: Not mentioned.

REVIEW OF SYSTEMS:

Constitutional: Reports fatigue, feeling "out of it," difficulty concentrating, and cognitive slowing.

ENT: Left-sided facial tingling, scalp numbness, abnormal sensation in ears.

Eyes: Blurry vision, abnormal visual phenomena when closing eyes.

Neurological: Loss of consciousness at time of MVC, persistent tingling, sensation of detachment, difficulty answering questions, subjective cognitive changes.

Musculoskeletal: Pain and bruising on left side (neck, leg, arm), difficulty ambulating.

Gastrointestinal: Nausea, gagging after Midazolam.

Skin: Bruising on left leg, arm, and knee.

PHYSICAL EXAM:

Constitutional: No acute distress

HENT: Normocephalic, atraumatic, oropharynx clear and moist

Eyes: PERRL, EOM normal, conjunctivae normal

Neck: CERVICAL PARASPINAL AND LEFT TRAPEZIUS TENDERNESS

Cardiovascular: Regular rhythm, heart sounds normal, intact distal pulses, cap refill <2 sec

Pulmonary: Effort normal, breath sounds normal, no respiratory distress, no chest tenderness

Abdominal: Soft, bowel sounds normal, no distension, no tenderness

Musculoskeletal: BRUISING ON LEFT LEG, ARM, AND KNEE; Range of motion normal, no edema, no deformity,

no tenderness

C-spine: no bony tenderness

T-spine: no bony tenderness

L-spine: no bony tenderness

Neurological: ALERT AND ORIENTED, NO FACIAL ASYMMETRY, PEOMI, NO EXTREMITY DRIFT, GROSS SENSATION INTACT, ABLE TO FOLLOW COMMANDS, ABLE TO LIFT EYEBROWS AND SQUEEZE EYES SHUT, ABLE TO HOLD ARMS OUTSTRETCHED

MS: Intact memory, concentration, attention, general fund of knowledge, orientation, language.

CN: PEERL, EOMI, VF full, Face intact motor and sensation, symmetrical uvular lift and tongue protrusion, SCM and trap 5/5 bilaterally.

MOT: NI bulk and tone.

Strength

Right UE: 5/5. Left UE: 5/5

Right LE: 5/5. Left LE: 5/5

SENS: In tact to light touch

COOR: no dysmetria, no dysdiadokokinesia

Gait: narrow based, no ataxia

Skin: Warm, dry

Psych: Normal affect

EVALUATIONS:

INITIAL EVALUATION AND PLAN:

Differential diagnosis includes but is not limited to: anemia, electrolyte abnormality, AKI, ICH, fracture, dislocation

- Differential diagnosis: Post-MVC contusions, muscle

strain/spasm, cervical strain, concussion, rule out intracranial hemorrhage or vascular injury (dissection), possible anxiety, evaluate for dehydration, renal insufficiency, electrolyte abnormality, or anemia.

- Plan:

- Repeat imaging of head and neck (CT)
- Pain management
- Reassessment after treatment
- Monitor for neurologic changes
- Laboratory evaluation for dehydration, renal function, electrolytes, and anemia

MEDICAL DECISION-MAKING:

Differential Diagnoses:

Post-MVC contusions, muscle strain/spasm, cervical strain, concussion, rule out intracranial hemorrhage or vascular injury (dissection), possible anxiety, evaluate for dehydration, renal insufficiency, electrolyte abnormality, or anemia.

Additional Historians:

- Family member: An independent historian (family member) corroborates the patient's cognitive changes and increased difficulty with ambulation and answering questions, noting that these symptoms worsened throughout the day.

Reviewed Notes:

- Chart review from Baylor Hospital: Chart review from

Baylor Hospital, 2 days prior, CTs (head, cervical spine, chest, abdomen, pelvis, thoracic and lumbar spine), x-rays of both knees, reportedly without acute findings.

Test Interpretations:
- I [independently interpreted / reviewed] the Head CT.
Results: [no hemorrhage]

Procedures

Data

Labs:
Lab Results

Procedu re	Compone nt	Value	Ref Range	Date/Time
CBC WITH DIFFERENTIAL				Collected:
[1780691769] (Abnormal)				08/30/26 2009
Specimen: Blood, Venous				Updated:
				08/30/26 2030
	WBC	10.97^	3.90 - 10.90 k/uL	
	RBC	4.83	3.70 - 5.50 M/uL	
	Hgb	14.4	11.0 -	

		16.3 g/dL
Hct	42.9	33.0 -
		47.0 %
MCV	88.8	75.0 -
		100.0 fL
MCH	29.8	27.0 -
		35.0 pg
MCHC	33.6	31.0 -
		37.0 g/dL
RDW -	12.7	11.7 -
CV		15.2 %
RDW -	41.6	fL
SD		
Platelet	342	150 - 450
Count		k/uL
MPV	10.2	9.4 - 12.3
		fL
NRBC	0.00	k/uL
abs		
NRBC %	0.00	%
Neutrop	68.2	%
hil %		
Lympho	22.5	%
cyte %		
Monocyt	5.5	%
e %		
Eosinop	3.0	%
hil %		
Basophil	0.5	%

%		
Imm	0.3	0.0 - 3.0
Grans %		%
Neutrop	7.48^	2.02 -
hil abs		7.46 k/uL
Lympho	2.47	1.01 -
cyte abs		3.38 k/uL
Monocyt	0.60	0.00 -
e abs		0.82 k/uL
Eosinop	0.33	0.00 -
hil abs		0.52 k/uL
Basophil	0.06	0.00 -
abs		0.09 k/uL
Imm	0.03	0.00 -
Grans		0.07 k/uL
abs		
Differenti	Auto	
al Type		

**COMPREHENSIVE METABOLIC
PANEL [1780691764] (Abnormal)**

Specimen: Blood, Venous

Collected:
08/30/26 2009
Updated:
08/30/26 2032

Sodium	141	136 - 145 mmol/L
Potassiu	4.0	3.4 - 4.5 mmol/L
m		
Chloride	102	101 - 110 mmol/L
CO2	26	20 - 31

Glucose	138[^]	mmol/L 70 - 100
BUN	14.2	mg/dL 6.0 - 20.0
Creatinin e	0.95	mg/dL 0.60 - 1.10
Calcium	9.0	mg/dL 8.6 - 10.0
Total Protein	7.9	g/dL 6.4 - 8.5
Albumin	4.4	g/dL 3.7 - 4.9
Alk Phos	91	U/L 40 - 150
AST (SGOT)	23	U/L 15 - 48
ALT (SGPT)	<12	U/L 8 - 56
Bilirubin, Total	0.3	mg/dL 0.2 - 1.0
eGFR	74	>=60 mL/min/1. 73m*2
AGap	13[^]	3 - 11
A/G Ratio	1.3	1.0 - 1.8
Osmolali	295	278 - 301

ty calc mOsm/K
g

BUN/Cre 14.9
at Ratio

Globulin 3.5 2.5 - 4.1
s

MAGNESIUM [1780691766] (Normal) Collected:
08/30/26 2009

Specimen: Blood, Venous Updated:
08/30/26 2032

Magnesi 1.8 1.3 - 2.1
um mEq/L

PREGNANCY TEST, SERUM Collected:
[1780691767] (Normal) 08/30/26 2009

Specimen: Blood, Venous Updated:
08/30/26 2027

Pregnan Negative Negative
cy Test, e
serum

Radiology: Imaging Results

CTA Head & Neck Angio Combo
(CTA HEAD NECK ANGIO

Result time 08/30/26

Final result

Impression:

IMPRESSION:

1. No acute intracranial abnormality.
2. No large vessel occlusion, dissection, aneurysm, or significant stenosis involving the major cervical or intracranial arteries.
3. There are multiple findings suggestive of chronic idiopathic intracranial hypertension, which may explain the history of headache.

End of report

Electronically signed by David Barton, DO at
8/30/2026 8:53 PM

Narrative:

EXAMINATION: CT HEAD WO CON, CTA HEAD
NECK ANGIO COMBO
REPORT DATE: 8/30/2026 8:41 PM

ORDERING PHYSICIAN: SHANNON KELSEY

DELLA MONICA

HISTORY: mvc, severe headache.

COMPARISON(S): CT head without contrast 10/15/2025; CTA head/neck 8/17/2023.

TECHNIQUE: CT of the head without contrast. CT angiogram of the head and neck after administration of intravenous iodinated contrast. This examination was performed according to

ALARA principles. The estimated degree of maximum luminal diameter stenosis is according to the NASCET standard. 3D and/or MIP reformatted images were provided per CTA protocol.

FINDINGS:

CT HEAD:

Brain: Brain parenchymal attenuation and morphology are normal. No acute hemorrhage. Gray-white matter differentiation is maintained. No mass effect or midline shift.

Ventricles/CSF spaces: Unchanged partially empty sella. Otherwise the ventricles and extra-axial CSF spaces appear normal.

CTA NECK:

Aorta: There is no visualized aortic aneurysm or dissection. There is a four-vessel origin of the aortic arch with the left vertebral artery arising directly from the aorta. The great vessel origins are normal.

Right Carotid: No occlusion, stenosis, or dissection of the common carotid and cervical internal carotid arteries.

Left Carotid: No occlusion, stenosis, or dissection of the common carotid and cervical internal carotid arteries.

Vertebral Arteries: The origins of the vertebral arteries are patent. There is no vertebral artery occlusion, dissection, or significant stenosis

CTA HEAD

Anterior Circulation: Normal in course and caliber. No high-grade stenosis or occlusion. No aneurysm or vascular malformation.

Posterior Circulation: Normal in course and caliber. No high-grade stenosis or occlusion. No aneurysm or vascular malformation.

OTHER:

Orbits: There is mild cortical tortuosity of the optic nerves with prominent CSF in the optic nerve sheaths. Otherwise normal.

Paranasal sinuses: Imaged paranasal sinuses are clear.

Mastoids/middle ears: Clear.

Calvarium/skull base: No acute osseous abnormality of the calvarium or skull base.

Spine: No acute cervical spine fracture or suspicious osseous lesion. There is mild/moderate appearing multilevel cervical spondylosis without any high-grade osseous narrowing of the cervical spinal canal or neural foramina.

Scalp/facial soft tissues: The paraspinal muscles are unremarkable. The parotid and submandibular glands are normal. The thyroid gland is normal. No cervical lymphadenopathy.

Lung apices: The imaged lung apices are clear.

CT Head, WO IV Contrast (CT
HEAD WO CON) (Final result)

Result time 08/30/26
20:53:07

Final result

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Lung apices: The imaged lung apices are clear.

Clinical Decision Tools:

415 - Emergency Medicine: Utilization of CT for Minor Blunt Head Trauma (Adult)

Patient is 18 or older, presenting with minor blunt head trauma. Head CT (including cosigned orders) was ordered by an emergency care clinician for trauma because (select one or more):

Patient has severe headache

Medications received in ED:

ED Medication Administration from 08/30/2026 1942 to 08/30/2026 2215

Date/Time	Order	Dose	Route	Action
08/30/2026 2010 CDT	NORMAL SALINE 1,000 mL	--	IV BO LU S	New Bag/Vial
08/30/2026 2009 CDT	fentaNYL PF (Sublimaze) solution	50 mcg	SL O W IV PU SH	Given
08/30/2026 2009 CDT	ondansetron (Zofran) solution	4 mg	IN TR AV EN OUS	Given
08/30/2026 2015 CDT	NORMAL SALINE solution	50 mL		New Bag/Vial
08/30/2026	iohexol (Omnipaque-350)	75 mL		Given

2015 **solution**
 CDT
 08/30/2 **ketorolac** 15 IN Given
 026 **(Toradol) solution** mg TR
 2138 AV
 CDT EN
 OU
 S

Follow-Up Information

Follow up With	Specialties	Details	Why	Contact Info
Uddin, Sarah		In 3 days		19 Yawpo Avenue Oakland NJ 07436 201-337- 9600
Texas Health Frisco Emergency Departmen	Emergency Room		As needed, If symptoms worsen	12400 N. Dallas Parkway Frisco Texas

t

75033
469-495-
2070

There are no referrals from this visit

	08/30/26 1945	08/30/26 2059	08/30/26 2140
BP:	155/99	141/82	109/68
Pulse:	98	72	70
Resp:	18	17	16
Temp:	(!) 96.1 °F (35.6 °C)	98.1 °F (36.7 °C)	97.2 °F (36.2 °C)
SpO2:	95%	96%	96%
Weight:	97.5 kg (215 lb)		
Height:	5' 6" (167.6 cm)		

Disposition

ED Disposition

ED	Condi	Comment
Disposi	on	--
tion	Good	
Discharge		

Home

Discharge Medication List as of 8/30/2026 9:45 PM

Discharge Medication List as of 8/30/2026 9:45 PM

ED Course as of 08/31/26 2321

Sun Aug 30, 2026

213 Results discussed with the patient. She
1 remains neurovascularly intact, ambulates with a steady gait, counseled on continued symptom control and given strict return precautions. Questions were answered. Patient was given follow-up instructions and counseled on the importance. Patient was instructed to return immediately to the emergency department if their symptoms did not improve or worsened. Patient verbalized understanding and agreement with this plan.

Encounter Diagnoses

Name

ED Diagnoses

Diagnosis	Comment	Time	
		Admitted By	Admitted

Final diagnoses

Motor vehicle collision, subsequent encounter	--	Delila Monica, Shannon Kelsey, MD	08/30/2026 2131
Strain of neck muscle, subsequent encounter	--	Delila Monica, Shannon	08/30/2026 2131

		Kel
		sey
		,
		MD
Acute	--	Del 08/
nonintr		la 30/
actable		Mo 202
headac		nic 6
he,		a, 213
unspeci		Sh 1
fied		ann
headac		on
he type		Kel
		sey
		,
		MD

Electronically signed by:
Shannon Della Monica, MD
Emergency Medicine Physician
Mediserv #698

